

Roche Group development pipeline

Marketed products development programmes

Roche Pharma global development programmes

Roche Pharma research and early development (pRED)

Genentech research and early development (gRED)

Changes to the development pipeline

Q1 2026 update

New to phase I	New to phase II	New to phase III	New to registration
5 NMEs: RG6627 APOE ASO - Alzheimer's disease RG6622 NME - coronary artery disease RG6858 NME - retinal diseases RG6981 IL33:TSLP – COPD/asthma RG6735 NME - heme tumors	1 NME: RG6182 MAGL inhibitor - multiple sclerosis 1 AI: RG6114 Itovebi + enzalutamide - CRPC	1 NME: RG6640 enicepatide (CT-388) - obesity +/- T2D	1 AI: RG7159 Gazyva - systemic lupus erythematosus 3 AIs (US): RG6171 giredestrant - ER+ BC adj RG7159 Gazyva - childhood onset idiopathic nephrotic syndrome RG7828 Lunsumio + Polivy - 2L+ DLBCL
Removed from phase I	Removed from phase II	Removed from phase III	Approvals
3 NMEs: RG6076 englumafusp alfa combos - heme tumors RG6377 Anti-leukocyte trafficking antibody – IBD CHU BRY10 - chronic diseases	3 NMEs: RG6237 emugrobart (GYM 329) + Evrysdi – SMA RG6287 flizaseritib - CS-AKI RG7816 alogabat - Angelman syndrome 2 AIs: RG6237 emugrobart (GYM 329) - FSHD RG6168 Enspryng - DMD	1 AI: RG6171 giredestrant + palbociclib - 1L ET sensitive ER+/HER2-mBC	

Roche Group development pipeline

Phase I (41 NMEs + 6 AIs)

RG6026	Columvi monotherapy + combos	heme tumors	RG6382	CD19 x CD3	SLE
RG6160	cevostamab	r/r multiple myeloma	RG6418*	selnoflast	inflammation
RG6171	giredestrant monotherapy + combos	solid tumors	RG6421	TMEM16A potentiator	Muco-obstructive respiratory disease
RG6330	divarasib monotherapy + combos	solid tumors	RG6498	-	IBD
RG6344	mosperafenib (BRAF inhibitor (3))	solid tumors	RG6631	afimkibart (anti-TL 1A)	MASH
RG6411	enzelkitug (anti-CCR8)	solid tumors	RG6981⁴	IL33:TSNP	COPD/Asthma
RG6468	-	solid tumors	RG7828	Lunsumio	SLE
RG6505	PanRAS inhibitor	solid tumors	CHU	anti-HLA-DQ2.5 x gluten peptides	celiac disease
RG6537	AR degrader	mCRPC	CHU	anti-C1s recycling antibody	immunology
RG6538	P-BCMA-ALLO1	r/r multiple myeloma	RG6622	-	coronary artery disease
RG6540	P-CD19 x CD20 - ALLO1	heme tumors	RG6035	Brainshuttle™ CD20	multiple sclerosis
RG6596	HER2 TKI	HER2+ BC	RG6434	-	neurodegenerative disorders
RG6620	KRAS G12D inhibitor	solid tumors	RG6496⁵	HTT SNP ASO	Huntington's Disease
RG6648¹	cMET ADC	solid tumors	RG6627⁵	APOE ASO	Alzheimer's disease
RG6735	NME	heme tumors	RG6662	HTT miRNA GT	Huntington's Disease
RG6919¹	B7H3 ADC	solid tumors	RG6209	VEGF x IL6 DutaFab	DME
RG7828	Lunsumio monotherapy + combos	heme tumors	RG6327	-	geographic atrophy
RG6794	CDK4/2i	HR+ HER2- BC	RG6858	-	retinal diseases
RG6795	CDK4i	HR+ HER2- BC	RG6913	-	DME
RG6810²	DLL3 ADC	SCLC	RG6006	zosurabalpin	bacterial infections
RG6975³	CDH17 ADC	CRC	CHU	REV24	acute diseases
CHU	clesitamig (DLL3 trispecific)	solid tumors			
CHU	codrituzumab	HCC			
CHU	MINT91	solid tumors			
CHU	anti-CTLA-4 switch antibody	solid tumors			
CHU	pan-KRAS inhibitor (AUBE00)	solid tumors			

Phase II (12 NMEs + 6 AIs)

RG6114	Itovebi + enzalutamide	CRPC
RG6171	giredestrant	endometrial cancer
RG6180	autogene cevumeran	solid tumors
RG6797	dirloctocogene samoparvovec (SPK-8011QQ)	hemophilia A
RG6512	NXT007	hemophilia
RG6631	afimkibart (anti-TL 1A)	atopic dermatitis
RG6631	afimkibart (anti-TL 1A)	rheumatoid arthritis
RG6730⁶	p40xTL 1A bispecific antibody	UC
RG6237	emugrobarb (GYM 329)	obesity
RG6641	acmopatide (CT-868)	T1D glycemic control
RG6849⁷	petrelintide	obesity +/- T2D
RG6652	GLP-1 RA (CT-996)	obesity/ T2D glycemic control
RG6042⁵	tominersen	Huntington's Disease
RG6182	MAGL inhibitor	multiple sclerosis
RG6289	nivegaceter (gamma-secretase modulator)	Alzheimer's
RG6356⁸	Elevidys	0 to <4 year old DMD
RG6351	Tie2 agonist	DME
RG6501	OpRegen	geographic atrophy

RG-No - Roche/Genentech; CHU - Chugai managed; ¹MediLink managed; ²Innovent managed; ³In collaboration with Hansoh Pharma, RG Nr TBD; ⁴Qyuns Therapeutics managed; ⁵In collaboration with IONIS; ⁶In collaboration with Pfizer; ⁷Zealand Pharma managed; ⁸US rights with Sarepta; *also developed in neurology

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology

Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

Roche Group development pipeline

Phase III (11 NMEs + 22 AIs)

RG3502	Kadcyla + T	HER-2+ eBC high-risk	RG6149	astegolimab	COPD
RG6013	Hemlibra	Type 3 VWD	RG6299²	sefaxersen (ASO factor B)	IgA nephropathy
RG6026	Columvi + Polivy + R-CHP	1L DLBCL	RG6631	afimkibart (anti-TL 1A)	ulcerative colitis
	Columvi	r/r MCL		afimkibart (anti-TL 1A)	Crohn's disease
RG6107	PiaSky	aHUS	RG7159	Gazyva	membranous nephropathy
RG6114	Itovebi + fulvestrant	post CDKi HR+ PIK3CA-mut. BC	RG6615¹	zilebesiran	hypertension
	Itovebi + Phesgo	1L HER2+ PIK3CA-mut. mBC	RG6640	enicepatide (CT-388)	obesity +/- T2D
	Itovebi + CDK4/6i + letrozole	1L ES PIK3CA-mut. HR+ HER2- advanced BC	RG6881	pegozafermin	MASH with fibrosis stage F2 or F3
	giredestrant + Phesgo	1L ER+/HER2+ BC		pegozafermin	MASH with fibrosis stage F4
RG6171	giredestrant + CDK4/6i	1L ET resistant ER+/HER2- BC	RG6102	trontinemab	early Alzheimer's
RG6330	divarasib	2L NSCLC	RG6168	Enspryng	MOGAD
RG6330	divarasib + pembrolizumab	1L NSCLC		Enspryng	autoimmune encephalitis
RG7828	Lunsumio + lenalidomide	2L+ FL	RG6356³	Elevidys	amb. 8 to <18y & non amb. DMD
			RG7845	fenebrutinib	RMS
				fenebrutinib	PPMS
			RG7935	prasinezumab	Parkinson's
			RG6168	Enspryng	TED
			RG6179	vamikibart	UME
			RG6321	Susvimo	wAMD, 36-week
			RG7716	Vabysmo	CNV

Registration US & EU (2 NMEs + 5 AIs)

RG6171	giredestrant + everolimus [†]	post-CDK4/6 ER+/HER2- BC
	giredestrant ^{††}	ER+ BC adj
RG7446	Tecentriq	ctDNA+ high-risk MIBC
RG7828	Lunsumio + Polivy ^{††}	2L+ DLBCL
RG7159	Gazyva	systemic lupus erythematosus
	Gazyva ^{††}	childhood onset idiopathic nephrotic syndrome*
RG6321	Susvimo ^{†††}	wAMD

[†]Filed in US

^{††}Filed in US, FDA acceptance pending

^{†††}Filed in EU, approved in US

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology

Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

Expected regulatory submissions*

New Molecular Entities: Lead and additional indications

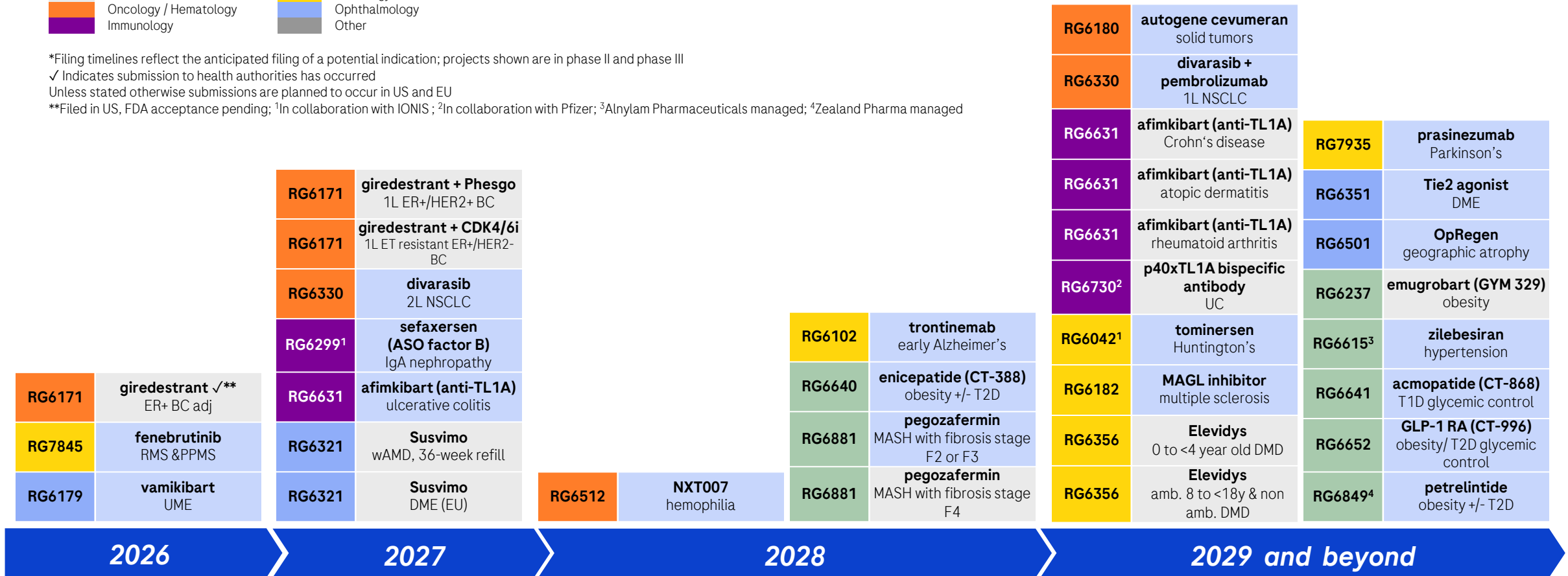
New Molecular Entity (NME)	Cardiovascular, Renal & Metabolism
Additional Indication (AI)	Neurology
Oncology / Hematology	Ophthalmology
Immunology	Other

*Filing timelines reflect the anticipated filing of a potential indication; projects shown are in phase II and phase III

✓ Indicates submission to health authorities has occurred

Unless stated otherwise submissions are planned to occur in US and EU

**Filed in US, FDA acceptance pending; ¹In collaboration with IONIS; ²In collaboration with Pfizer; ³Alnylam Pharmaceuticals managed; ⁴Zealand Pharma managed



Expected regulatory submissions*

Marketed products: Additional indications

New Molecular Entity (NME)	Cardiovascular, Renal & Metabolism
Additional Indication (AI)	Neurology
Oncology / Hematology	Ophthalmology
Immunology	Other

*Filing timelines reflect the anticipated filing of a potential indication; projects shown are in phase II and phase III

✓ Indicates submission to health authorities has occurred

Unless stated otherwise submissions are planned to occur in US and EU

FDA acceptance pending; *also known as pediatric nephrotic syndrome (PNS)

		RG7159	Gazyva membranous nephropathy	RG6013	Hemlibra Type 3 VWD		
		RG7159	Gazyva ✓ systemic lupus erythematosus	RG6114	Itovebi + Phesgo 1L HER2+ PIK3CA-mut. mBC		
RG6107	PiaSky aHUS	RG7159	Gazyva ✓ childhood onset idiopathic nephrotic syndrome***	RG3502	Kadcyla + Tecentriq HER-2+ eBC high-risk		
RG6114	Itovebi + fulvestrant post CDKi HR+ PIK3CA-mut. BC	RG6168	Enspryng MOGAD	RG6026	Columvi + Polivy + R-CHP 1L DLBCL		
RG7828	Lunsumio + lenalidomide 2L FL+	RG6168	Enspryng TED	RG6026	Columvi r/r MCL	RG6114	Itovebi + CDK4/6i + letrozole 1L ES PIK3CA-mut. HR+ HER2- advanced BC
RG7828	Lunsumio + Polivy ✓** 2L+ DLBCL (US)	RG7716	Vabysmo CNV	RG6168	Enspryng autoimmune encephalitis	RG6114	Itovebi + enzalutamide CRPC
2026				2027		2028	
						2029 and beyond	

Major pending approvals 2026

US		EU		China	Japan-Chugai	
RG6171	giredestrant + everolimus post-CDK4/6 ER+/HER2- BC Filed Dec 2025	RG6321	Susvimo wAMD Filed July 2025	RG6171	giredestrant ER+ BC adj Filed April 2026	RG7853 Alecensa ALK+ solid tumors Filed June 2025
RG7446	Tecentriq ctDNA+ high-risk MIBC Filed Nov 2025	RG7159	Gazyva systemic lupus erythematosus Filed Feb 2026		RG435 Avastin Neurofibromatosis Type 2 Filed August 2025	
RG7159	Gazyva systemic lupus erythematosus Filed Feb 2026				RG7446 Tecentriq Adjuvant Therapy for MRD-Positive Bladder Cancer Filed January 2026	
RG6171	giredestrant* ER+ BC adj Filed March 2026					
RG7159	Gazyva* childhood onset idiopathic nephrotic syndrome** Filed March 2026					
RG7828	Lunsumio + Polivy* 2L+ DLBCL Filed April 2026					

*FDA acceptance pending, **also known as pediatric nephrotic syndrome (PNS)

Status as of April 23, 2026

	New Molecular Entity (NME)		Cardiovascular, Renal & Metabolism
	Additional Indication (AI)		Neurology
	Oncology / Hematology		Ophthalmology
	Immunology		Other

Major granted approvals 2026

US	EU	China	Japan-Chugai
		RG7596 Polivy + chemo r/r DLBCL March 2026	RG7828 Lunsumio + Polivy r/r large B-cell lymphoma March 2026

- New Molecular Entity (NME)

Additional Indication (AI)

Oncology / Hematology

Immunology
- Cardiovascular, Renal & Metabolism

Neurology

Ophthalmology

Other

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Marketed products development programmes

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Genentech research and early development (gRED)

Marketed products development programmes

Oncology / Hematology		Neurology		Immunology		Ophthalmology		CVRM	
RG7853	Alecensa	RG6356	Elevidys	RG7159	Gazyva	RG6168	Enspryng		
RG6114	Itovebi	RG6168	Enspryng	RG7828	Lunsumio	RG6321	Susvimo		
RG3502	Kadcyla	RG7916	Evrysdi			RG7716	Vabysmo		
RG7446	Tecentriq								
RG6026	Columvi								
RG7828	Lunsumio								
RG7596	Polivy								
RG6013	Hemlibra								
RG6107	PiaSky								

Alecensa (alectinib, RG7853)

CNS-active inhibitor of anaplastic lymphoma kinase (ALK)

Indication	Locally advanced, unresectable, stage III NSCLC
Phase/study	Phase III HORIZON-01
# of patients	N=70
Design	▪ ARM A: Alecensa 600mg BID ▪ ARM B: durvalumab 1500 Q4W
Primary endpoint	▪ Progression-free survival
Status	▪ FPI Q4 2022 ▪ Recruitment completed Q2 2025
CT Identifier	NCT05170204

Itovebi (inavolisib, RG6114, GDC-0077)

A potent, orally available, and selective PI3K α inhibitor

Indication	PIK3CA-mutant HR-positive metastatic breast cancer (mBC)	post CDKi HR-positive PIK3CA-mutant breast cancer
Phase/study	Phase III INAVO120	Phase III INAVO121
# of patients	N=320	N=400
Design	ARM A: Itovebi + palbociclib + fulvestrant ARM B: Placebo + palbociclib + fulvestrant	ARM A: Itovebi + fulvestrant ARM B: alpelisib + fulvestrant
Primary endpoint	Progression-free survival	Progression-free survival
Status	- FPI Q1 2020 - Recruitment completed Q3 2023 - Study met its primary endpoint of PFS Q4 2023 - Data presented at SABCS 2023 - BTD granted by FDA Q2 2024 - Filed in US (priority review) and EU Q2 2024 - Data presented at ASCO 2024 and ASCO 2025 - Approved in US Q3 2024 - Published in NEJM 2024;391:1584-1596 and NEJM 2025;393:151-161 - Approved in EU July 2025	- FPI Q2 2023 - Recruitment completed Q4 2024
CT Identifier	NCT04191499	NCT05646862

ASCO: American Society of Clinical Oncology; HR: Hormone receptor; HER2: Human Epidermal growth factor Receptor 2; NEJM: New England Journal of Medicine; PIK3CA-mut: phosphatidylinositol 3-kinase, catalytic, alpha polypeptide mutated; SABCS: San Antonio Breast Cancer Symposium

Itovebi (inavolisib, RG6114, GDC-0077)

A potent, orally available, and selective PI3K α inhibitor

Indication	1L HER2-positive PIK3CA mutant metastatic breast cancer (mBC)	1L endocrine-sensitive PIK3CA-mutated HR+, HER2-, advanced breast cancer
Phase/study	Phase III INAVO122	Phase III INAVO123
# of patients	N=230	N=450
Design	ARM A: Itovebi + Phesgo after induction therapy with Phesgo + taxane ARM B: Placebo + Phesgo after induction therapy with Phesgo + taxane	ARM A: Itovebi + CDK4/6i + letrozole ARM B: Placebo + CDK4/6i + letrozole
Primary endpoint	Progression-free survival	Progression free survival
Status	FPI Q3 2023	FPI Q2 2025
CT Identifier	NCT05894239	NCT06790693

HR: Hormone receptor; HER2: Human Epidermal growth factor Receptor 2; PIK3CA-mut: Phosphatidylinositol 3-kinase, catalytic, alpha polypeptide mutated; Phesgo: FDC of Perjeta and Herceptin for SC administration with Halozyme's rHuPH20/ Halozyme's human hyaluronidase

Itovebi (inavolisib, RG6114, GDC-0077)

A potent, orally available, and selective PI3K α inhibitor

Indication	post-ARPI metastatic prostate cancer (mCRPC)	
Phase/study	Phase II InavoPC	
# of patients	N=100	
Design	▪ ARM A: Itovebi + enzalutamide ▪ ARM B: ARPI switch (enzalutamide or abiraterone) or docetaxel	
Primary endpoint	▪ Radiographic Progression-free Survival (rPFS)	
Status	▪ FPI Q1 2026	
CT Identifier	NCT07287150	

PIK3CA-mut: Phosphatidylinositol 3-kinase, catalytic, alpha polypeptide mutated

Kadcyla (trastuzumab emtansine, RG3502)

First ADC for HER2-positive breast cancer

Indication	HER2-positive early breast cancer (BC) high-risk patients
Phase/study	Phase III ASTEFANIA
# of patients	N=1150
Design	- ARM A: Kadcyla + Tecentriq - ARM B: Kadcyla + placebo
Primary endpoint	Invasive disease-free survival
Status	- FPI Q2 2021 - Recruitment completed Q4 2024
CT Identifier	NCT04873362

In collaboration with Abbvie
 ADC: antibody drug conjugate; HER2: Human Epidermal growth factor Receptor 2

Tecentriq (atezolizumab, RG7446)

Anti-PD-L1 cancer immunotherapy – lung cancer

Indication	1L maintenance extensive-stage SCLC
Phase/study	Phase III IMforte ¹
# of patients	N=450
Design	▪ ARM A: Platinum-etoposide + Tecentriq followed by maintenance Tecentriq + lurbinectedin ▪ ARM B: Platinum-etoposide + Tecentriq followed by maintenance Tecentriq
Primary endpoint	▪ Progression-free survival and overall survival
Status	▪ FPI Q4 2021 ▪ Recruitment completed Jan 2024 ▪ Study met primary endpoints Q3 2024 ▪ Filed in US (priority review) Q2 2025 ▪ Data presented at ASCO 2025 ▪ Approved in US Q3 2025
CT Identifier	NCT05091567

¹In collaboration with Jazz Pharma

SCLC: Small cell lung cancer; PD-L1: Programmed cell death-ligand 1; ASCO: American Society of Clinical Oncology

Tecentriq (atezolizumab, RG7446)

Anti-PD-L1 cancer immunotherapy – urothelial carcinoma

Indication	ctDNA+, high-risk muscle-invasive bladder cancer (MIBC)
Phase/study	Phase III IMvigor011
# of patients	N=240
Design	▪ ARM A: Tecentriq monotherapy ▪ ARM B: Placebo
Primary endpoint	▪ Disease-free survival (DFS)
Status	▪ FPI Q2 2021 ▪ Recruitment completed Q2 2025 ▪ Study met primary point of DFS Q3 2025 ▪ Data presented at ESMO 2025 ▪ Filed in US Q4 2025
CT Identifier	NCT04660344

ESMO: European Society for Medical Oncology; PD-L1: Programmed cell death-ligand 1

Columvi (glofitamab, RG6026)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	Relapsed or refractory Non-Hodgkin's lymphoma (NHL)	
Phase/study	Phase I	Phase I
# of patients	N=700	N=18-36
Design	Cohort 1: Single-agent dose escalation study - Initial dose escalation - Expansion cohort in r/r DLBCL - Expansion cohort in r/r FL - Expansion cohort in r/r MCL - All patients will receive pretreatment with a single dose of Gazyva (1000mg) Cohort 2: Columvi + Gazyva (i.e. continuous treatment with Gazyva)	Columvi SC Part 1 dose escalation
Primary endpoint	Efficacy, safety, tolerability and PK	Safety
Status	- Data presented at ASH 2018, 2020, 2021, 2022, 2023, ICML 2019, 2021, EHA 2020, 2021, 2022 and ASCO 2021, 2022 and 2023 - Data published in <i>J Clin Oncology</i> 2021; 39:18:1959-1970 and <i>NEJM</i> 2022; 387:2220-2231 - Filed in EU Q2 2022 and US Q4 2022 - Approved in Canada Q1, US Q2 and EU Q3 2023	- FPI Q3 2021 - Recruitment completed Q1 2024
CT Identifier	NCT03075696	ISRCTN17975931

DLBCL: Diffuse large B cell lymphoma; FL: Follicular lymphoma; r/r: Relapsed or refractory; PK: Pharmacokinetics; ASCO: American Society of Clinical Oncology; ASH: American Society of Hematology; EHA: European Hematology Association; ICML: International Conference on Malignant Lymphoma; NEJM: New England Journal of Medicine

Columvi (glofitamab, RG6026)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	2L+ SCT-ineligible DLBCL
Phase/study	Phase Ib
# of patients	N=112
Design	▪ ARM A: Columvi IV + CELMoD (CC-220 and CC-99282) ▪ ARM B: Lunsumio SC + CELMoD (CC-220 and CC-99282)
Primary endpoint	▪ Safety, DLT, RPTD
Status	▪ FPI Q3 2019
CT Identifier	NCT05169515

DLBCL: Diffuse large B cell lymphoma; DLT: Dose-limiting toxicity, RPTD: Recommended Phase II Dose

Columvi (glofitamab, RG6026)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	2L+ SCT-ineligible DLBCL	1L DLBCL fit (IPI 2-5)
Phase/study	Phase III STARGLO	Phase III SKYGLO
# of patients	N=270	N=1130
Design	ARM A: Columvi + gemcitabine and oxaliplatin, followed by up to 4 cycles of Columvi monotherapy ARM B: Rituximab in combination with gemcitabine and oxaliplatin A single dose of Gazyva will be administered 7 days prior to the first dose of Columvi	ARM A: Columvi + Polivy + R-CHP ARM B: Polivy + R-CHP
Primary endpoint	Overall survival	Progression-free survival
Status	- FPI Q1 2021 - Recruitment completed Q1 2023 - Study met primary endpoint April 2024 - Data presented at EHA 2024 - Filed in EU and US Q3 2024 - Approved in EU Q2 2025 2yr follow-up data presented at ASCO 2025 3yr follow-up data presented at ASH 2025	- FPI Q4 2023 - Recruitment completed Q3 2025
CT Identifier	NCT04408638	NCT06047080

Columvi (glofitamab, RG6026)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	Relapsed or refractory mantle cell lymphoma (MCL)
Phase/study	Phase III GLOBRYTE
# of patients	N=182
Design	▪ ARM A: Columvi monotherapy ▪ ARM B: Bendamustine + rituximab or rituximab + lenalidomide
Primary endpoint	▪ Progression-free survival by IRC
Status	▪ FPI Q4 2023 ▪ BTB granted by FDA Q2 2024
CT Identifier	NCT06084936

BTB: Breakthrough Therapy Designation; IRC: Independent review committee

Lunsumio (mosunetuzumab, RG7828)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	2L+ FL
Phase/study	Phase III CELESTIMO
# of patients	N=474
Design	▪ ARM A: Lunsumio + lenalidomide ▪ ARM B: Rituximab + lenalidomide
Primary endpoint	▪ Progression-free survival
Status	▪ FPI Q4 2021
CT Identifier	NCT04712097

FL: Follicular lymphoma

Lunsumio (mosunetuzumab, RG7828)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	2L+ SCT ineligible DLBCL	Relapsed or refractory LBCL & MCL
Phase/study	Phase III SUNMO	Phase Ib/II
# of patients	N=222	N=235
Design	ARM A: Lunsumio + Polivy ARM B: R + GemOx	Dose escalation of Lunsumio + Polivy ARM A: Lunsumio SC + Polivy ARM B: Rituximab + Polivy
Primary endpoint	Progression-free survival	Safety/tolerability and response
Status	- FPI Q2 2022 - Recruitment completed Q4 2024 - Study met dual primary endpoints (ORR, PFS) Q2 2025 - Data presented at EHA, ICML 2025 - Filed in US April 2026 (FDA acceptance pending)	- FPI Q3 2018 - Initial data presented at ASCO 2021 and ASH 2021, 2022 - Data presented at ASH 2023 - Data published in <i>Nature Medicine</i> 2023; 30, 229-239 - Recruitment completed Q1 2024
CT Identifier	NCT05171647	NCT03671018

ASCO: American Society of Clinical Oncology; ASH: American Society of Hematology; DLBCL: Diffuse large B cell lymphoma; EHA: European Hematology Association; ICML: International Conference on Malignant Lymphoma; MCL: Mantle cell lymphoma; LBCL: Large B cell lymphoma; R: Rituxan/MabThera; GemOx: Gemcitabin und Oxaliplatin

Polivy (polatuzumab vedotin, RG7596)

ADC targeting CD79b to treat B cell malignancies

Indication	1L DLBCL
Phase/study	Phase III POLARIX
# of patients	N=879
Design	▪ ARM A: Polivy + R-CHP ▪ ARM B: R-CHOP
Primary endpoint	▪ Progression-free survival
Status	▪ Data presented at ASH 2021 and 2022 and 2024 ▪ Filed in EU, Japan and China Q4 2021 and in the US Q3 2022 ▪ Published in <i>NEJM</i> 2022 27;386(4):351-363 ▪ Approved in EU Q2 2022, Japan Q3 2022, China Q1 2023 and US April 2023
CT Identifier	NCT03274492

In collaboration with Pfizer

ASH: American Society of Hematology, DLBCL: diffuse large B cell lymphoma; NEJM: New England Journal of Medicine; R-CHP: Rituxan, cyclophosphamide, hydroxydoxorubicin, prednisone; R-CHOP: Rituxan, cyclophosphamide, doxorubicin, vincristine, and prednisone

Hemlibra (emicizumab, RG6013)

Factor VIII mimetic for treatment of hemophilia A

Indication	Type 3 Von Willebrand disease (VWD)
Phase/study	Phase III WILL-EMI
# of patients	N=66
Design	Open-label in participants aged 2 years and above diagnosed with Type 3 von Willebrand disease (VWD) on prior SOC on demand therapy ▪ ARM A: Hemlibra prophylaxis ▪ ARM B: Continuation of SOC on-demand therapy ▪ ARM C: Participants on prior SOC prophylactic therapy (Hemlibra prophylaxis) - to be assessed via intra-participant analysis with data obtained from the preceding non-interventional study (NIS)
Primary endpoint	▪ Efficacy (Annualized Bleed Rate at 24w), safety, pharmacokinetics, and pharmacodynamics
Status	▪ FPI Q2 2025
CT Identifier	NCT06998524

In collaboration with Chugai; Note: Hemlibra autoinjector in development for existing and future potential Hemlibra indications

PiaSky (crovalimab, RG6107, SKY59)

A humanized monoclonal antibody against complement C5

Indication	Atypical hemolytic uremic syndrome (aHUS) - adults	Atypical hemolytic uremic syndrome (aHUS) - paediatric
Phase/study	Phase III COMMUTE-a	Phase III COMMUTE-p
# of patients	N=90	N=35
Design	Single-arm study of aHUS patients ▪ Cohort 1: not previously treated with C5i ▪ Cohort 2: switching from C5i ▪ Cohort 3: known C5 polymorphism	Single-arm study of aHUS patients ▪ Cohort 1: not previously treated with C5i ▪ Cohort 2: switching from C5i ≤18y/o ▪ Cohort 3: previously treated with C5i (includes participants with known C5 polymorphism)
Primary endpoint	▪ Cohort 1+3: proportion of patients with complete TMA response anytime between baseline and week 25 ▪ Cohort 2: proportion of patients with maintained TMA control from baseline through week 25	▪ Cohort 1: proportion of patients with complete TMA response anytime between baseline and week 25 ▪ Cohort 2: proportion of patients with maintained TMA control from baseline through week 25
Status	▪ FPI Q4 2021 ▪ Recruitment completed Q2 2025 ▪ Positive results Q4 2025	▪ FPI Q4 2021 ▪ Recruitment completed Q1 2025 ▪ Positive results Q4 2025
CT Identifier	NCT04861259	NCT04958265

In collaboration with Chugai

aHUS: Atypical Hemolytic Uremic Syndrome; C5i: C5 inhibitor; TMA: thrombotic microangiopathy

Elevidys (delandistrogene moxeparvovec, RG6356, SRP-9001)

rAAVrh74.MHCK7.Micro-dystrophin gene therapy

Indication	Duchenne muscular dystrophy (DMD)
Phase/study	Phase II ENVOL
# of patients	N=21
Design	Open label single arm study in 0 to <4 year old DMD boys who will receive a single intravenous (IV) infusion of Elevidys on Day 1, separated into 4 cohorts: ▪ Cohort A: ~ 10 participants who are 3 years of age ▪ Cohort B: ~ 4 participants who are 2 years of age ▪ Cohort C: ~ 4 participants who are > 6 months to < 2 years of age ▪ Cohort D: ~ 3 participants who are <= 6 months of age
Primary endpoint	▪ Safety
Status	▪ FPI Q4 2023 ▪ Cohort A: Recruitment completed Q3 2024
CT Identifier	NCT06128564

In collaboration with Sarepta
DMD: Duchenne muscular dystrophy

Enspryng (satralizumab, RG6168, SA237)

Anti-IL-6 receptor humanized monoclonal antibody

Indication	Myelin oligodendrocyte glycoprotein antibody disease (MOGAD)	Autoimmune encephalitis (AIE)
Phase/study	Phase III METEOROID	Phase III CIELO
# of patients	N=152	N=152
Design	ARM A: Enspryng at weeks 0, 2, 4 (loading doses) and maintenance doses Q4W ARM B: Placebo	ARM A: Enspryng at weeks 0, 2, 4 (loading doses) and maintenance doses Q4W ARM B: Placebo
Primary endpoint	Time from randomization to the first occurrence of a MOGAD relapse	Efficacy (proportion of participants with mRS score improvement ≥ 1 from baseline and no use of rescue therapy at week 24 (NMDAR) and week 52 (LGI1) and safety)
Status	- FPI Q3 2022 - ODD granted by FDA in Q4 2021 - Study met primary endpoint Jan 2026 - Data presented at AAN 2026	- FPI Q3 2022 - ODD granted for NMDAR AIE in US Q3 22 and for LGI1 AIE Q4 2024
CT Identifier	NCT05271409	NCT05503264

In collaboration with Chugai; MG-AD: Myasthenia Gravis Activities of Daily Living; MOGAD: Myelin Oligodendrocyte Glycoprotein Antibody Disease; AIE: Autoimmune encephalitis; NMDAR AIE: Anti-N-Methyl-D-Aspartic Acid Receptor Autoimmune Encephalitis; ODD: Orphan drug designation

Evrysdi (risdiplam, RG7916)

Oral SMN2 splicing modifier

Indication	Spinal muscular atrophy (SMA)	
Phase/study	Phase II RAINBOWFISH	Phase II JEWELFISH
# of patients	N=25	N=174
Design	Infants aged from birth to 6 weeks who have been genetically diagnosed with SMA but are not yet presenting with symptoms	Adult and pediatric patients with previously treated SMA type 1, 2 and 3
Primary endpoint	Proportion of participants with two copies of the SMN2 gene and baseline CMAP$\geq$1.5 millivolt who are sitting without support	Safety, tolerability, PK/PD
Status	- FPI Q3 2019 - Recruitment completed Q1 2022 - Initial data presented at CureSMA, WMS 2021, MDA and WMS 2022 - Primary data presented at WMS 2023 - Filed in US and EU Q4 2021 - Approved in US Q2 2022 and EU Q3 2023 2-year data presented at WMS 2024 2yr data published in NEJM 2025; 393:671-682 3-year data presented at WMS 2025	- Data presented at WMS 2017, AAN 2018, WMS 2018, CureSMA 2019, WMS 2019, CureSMA 2020 and 2021 2-year data presented at WMS 2022 - Data published J Neurol. 2024 Aug;271(8):4871-4884 5-year data presented at WMS 2025
CT Identifier	NCT03779334	NCT03032172

In collaboration with PTC Therapeutics and SMA Foundation

SMN: survival motor neuron; CMAP: compound muscle action potential; WMS: World Muscle Society; CureSMA: Annual SMA Conference; MDA: Muscular Dystrophy Association

Gazyva (obinutuzumab, RG7159)

Humanized anti-CD20 monoclonal antibody; immunology development program

Indication	Lupus nephritis	Childhood onset idiopathic nephrotic syndrome*
Phase/study	Phase III REGENCY	Phase III INShore
# of patients	N=252	N=80
Design	▪ ARM A: Gazyva 1000mg IV (6 doses through Week 52) + MFF ▪ ARM B: Gazyva 1000 mg IV (5 doses through Week 52) + MFF ▪ ARM C: Placebo IV + MFF	▪ ARM A: Gazyva + oral steroids ▪ ARM B: Mycophenolate mofetil (MMF) + oral steroids
Primary endpoint	▪ Percentage of participants who achieve complete renal response (CRR)	▪ Percentage of participants with sustained complete remission at 1 year
Status	▪ FPI Q3 2020 ▪ Recruitment completed Q1 2023 ▪ Primary endpoint met Q3 2024 ▪ Filed in US and EU in Q1 2025 ▪ Published in NEJM 2025 Feb 7. doi: 10.1056/NEJMoa2410965. ▪ Approved in US Q3 2025 and EU Q4 2025	▪ FPI Q1 2023 ▪ Recruitment completed Q3 2024 ▪ Primary endpoint met Q4 2025 ▪ Filed in US Q1 2026 (FDA acceptance pending) ▪ Data presented at WCN 2026
CT Identifier	NCT04221477	NCT05627557

In collaboration with Biogen

*also known as pediatric nephrotic syndrome (PNS); MMF: mycophenolate mofetil; WCN: World Congress of Nephrology

Gazyva (obinutuzumab, RG7159)

Humanized anti-CD20 monoclonal antibody; immunology development program

Indication	Systemic lupus erythematosus (SLE)	Membranous nephropathy
Phase/study	Phase III ALLEGORY	Phase III MAJESTY
# of patients	N=300	N=140
Design	ARM A: Gazyva 1000mg IV on Day 1 and Weeks 2, 24 and 26. ARM B: Placebo IV	ARM A: Gazyva 1000mg IV on top of renin-angiotensin inhibitors ARM B: Tacrolimus treatment for 12 months
Primary endpoint	Percentage of participants who achieve Systemic Lupus Erythematosus Responder Index (SRI) at week 52	Percentage of patients who achieve complete remission at week 104
Status	- FPI Q4 2021 - Recruitment completed Q3 2024 - Primary endpoint met Q4 2025 - Filed in US and EU Q1 2026 - Data presented at SLEuro 2026	- FPI Q2 2021 - Recruitment completed Q4 2023 - Primary endpoint met Q1 2026
CT Identifier	NCT04963296	NCT04629248

In collaboration with Biogen

Lunsumio (mosunetuzumab, RG7828, CD20 x CD3)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	Systemic lupus erythematosus (SLE)
Phase/study	Phase I
# of patients	N=15
Design	▪ ARM A: Lunsumio SC on either Day 1 or on Days 1 and 8 ▪ ARM B: Fractionated (divided) dose of mosunetuzumab SC on Days 1 and 8
Primary endpoint	▪ Safety
Status	▪ FPI Q1 2022 ▪ Recruitment completed Q3 2023 ▪ Data presented at EULAR 2025
CT Identifier	NCT05155345

In collaboration with Biogen
EULAR: European Alliance of Associations for Rheumatology

Enspryng (satralizumab, RG6168, SA237)

Anti-IL-6 receptor humanized monoclonal antibody

Indication	Thyroid eye disease	
Phase/study	Phase III SatraGO-1	Phase III SatraGO-2
# of patients	N=120	N=120
Design	ARM A: Enspryng at weeks 0, 2, 4 (loading doses) and maintenance doses Q4W ARM B: Placebo	ARM A: Enspryng at weeks 0, 2, 4 (loading doses) and maintenance doses Q4W ARM B: Placebo
Primary endpoint	Proportion of participants with active disease achieving ≥ 2 mm reduction in proptosis from baseline (Day 1) at week 24 in the study eye, provided there is no deterioration of proptosis (≥ 2mm increase) in the fellow eye	Proportion of participants with active disease achieving ≥ 2 mm reduction in proptosis from baseline (Day 1) at week 24 in the study eye, provided there is no deterioration of proptosis (≥ 2mm increase) in the fellow eye
Status	- FPI Q4 2023 - Recruitment completed Q1 2025 - SatraGO-1 did not meet its primary endpoint, but showed a positive numerical trend towards proptosis improvement - Data presented at ASOPRS 2025	- FPI Q4 2023 - Recruitment completed Q1 2025 - SatraGO-2 met its primary endpoint - Data presented at ASOPRS 2025
CT Identifier	NCT05987423	NCT06106828

In collaboration with Chugai
ASOPRS: American Society of Ophthalmic Plastic and Reconstructive Surgery

Susvimo (PDS, RG6321)

First eye implant to achieve sustained delivery of a biologic medicine

Indication	Wet age-related macular degeneration (wAMD)		
Phase/study	Phase III Archway	Phase II+III extension Portal	Phase IIIb Velodrome
# of patients	N=418	N=1,000	N=442
Design	ARM A: PDS 100mg/mL Q24W ARM B: Intravitreal ranibizumab Q4W	- Ex-LADDER/ex-Archway: PDS 100mg/mL Q24W - Ex-Velodrome, not meeting Q36W criteria @ week 24: PDS 100mg/mL Q24W - Ex-Velodrome (completed or withdrawn): PDS Q24W or Q36W (as per Velodrome randomization)	ARM A: PDS 100mg/mL Q36W ARM B: PDS 100mg/mL Q24W
Primary endpoint	Change in BCVA from baseline at the average of week 36 and week 40	Long term safety efficacy	Change in BCVA from baseline averaged over weeks 68 and 72
Status	- Study met primary endpoint Q2 2020 - Data presented at ASRS 2020, 44/48 week data at Angiogenesis 2021 and 2-year data at Angiogenesis 2022 - Approved in US Q4 2021 5 year data presented at ASRS 2025 7 year data presented to EURETINA 2025 - CONTIVUE® with Susvimo filed in EU Q3 2025	FPI Q3 2018	- FPI Q3 2021 - Recruitment completed Q3 2025
CT Identifier	NCT03677934	NCT03683251	NCT04657289

ASRS: American Society of Retinal Specialists; BCVA: Best corrected visual acuity; wAMD: Wet age-related macular degeneration; PDS: Port Delivery System with ranibizumab

Susvimo (PDS, RG6321)

First eye implant to achieve sustained delivery of a biologic medicine

Indication	Diabetic macular edema (DME)	Diabetic retinopathy (DR) without center-involved diabetic macular edema (DME)
Phase/study	Phase III Pagoda	Phase III Pavilion
# of patients	N=634	N=174
Design	ARM A: Intravitreal ranibizumab (X4) followed by PDS 100mg/mL Q24W ARM B: Intravitreal ranibizumab Q4W until PDS 100mg/mL is received	ARM A: Intravitreal ranibizumab (X2) followed by PDS 100mg/mL (refill Q36W) ARM B: Q4W comprehensive clinical monitoring (with IVT ranibizumab as needed) until participants receive PDS 100mg/mL (refill Q36W)
Primary endpoint	Change in BCVA from baseline at the average of week 60 and week 64	Percentage of participants with a ≥ 2-step improvement from baseline on the ETDRS-DRSS at Week 52
Status	- FPI Q3 2019 - Recruitment completed Q2 2021 - Study met its primary endpoint Q4 2022 - Data presented at Angiogenesis 2023 - Filed in US Q2 2024 2-year data presented at ASRS 2024 - Approved in US Q1 2025	- FPI Q3 2020 - Recruitment completed Q3 2021 - Study met its primary endpoint Q4 2022 - Data presented at Angiogenesis 2023 - Filed in US Q2 2024 2-year data presented at ASRS 2024 - Approved in US Q2 2025
CT Identifier	NCT04108156	NCT04503551

ASRS: American Society of Retinal Specialists; BCVA: Best corrected visual acuity; ETDRS: Early Treatment Diabetic Retinopathy Study; DRSS: Diabetic Retinopathy Severity Scale; PDS: Port Delivery System with ranibizumab

Vabysmo (faricimab, RG7716)

Bispecific antibody for the eye which targets and inhibits two signaling pathways (Ang-2 and VEGF-A)

Indication	Myopic choroidal neovascularization (CNV)
Phase/study	Phase III POYANG
# of patients	n=280
Design	▪ ARM A: Vabysmo 6.0 mg Q4W PRN ▪ ARM B: Ranibizumab 0.5 mg Q4W PRN
Primary endpoint	▪ Change from Baseline in Best-Corrected Visual Acuity (BCVA) Averaged Over Weeks 4, 8, and 12
Status	▪ FPI Q1 2024 ▪ Recruitment completed Q2 2025
CT Identifier	NCT06176352

Ang-2: Angiopoietin-2; VEGF: Vascular endothelial growth factor; PRN: Pro re nata

Roche Group development pipeline

Marketed products development programmes

Roche Pharma global development programmes

Roche Pharma research and early development (pRED)

Genentech research and early development (gRED)

Roche Pharma global development programmes

Oncology / Hematology		Neurology		Immunology		Ophthalmology		CVRM	
RG6330	Divarasib	RG7845	Fenebrutinib	RG6299	Sefaxersen	RG6179	Vamikibart	RG6640	Enicepatide (CT-388)
RG6171	Giredestrant	RG7935	Prasinezumab	RG6631	Afimkibart			RG6652	CT-996
RG6512	NXT007	RG6042	Tominersen					RG6641	Acmopatide (CT-868)
RG6797	Dirloctocogene Samoparvovec	RG6102	Trontinemab					RG6237	Emugrobart
								RG6881	Pegozafermin

Divarasib (KRAS G12C inhibitor, RG6330, GDC-6036)

A potent, orally available, and selective inhibitor of the KRAS G12C mutant protein

Indication	1L NSCLC	2L NSCLC	1L NSCLC
Phase/study	Phase Ib/II KRASCENDO 170	Phase III KRASCENDO 1	Phase III KRASCENDO 2
# of patients	N=320	N=320	N=600
Design	Cohort A: Combination of divarasib + pembrolizumab Cohort B: Combination of divarasib + pembrolizumab + carboplatin/cisplatin + pemetrexed	- H2H vs KRAS G12Ci ARM A: divarasib ARM B: locally available G12Ci (sotorasib or adagrasib)	ARM A: divarasib + pembrolizumab ARM B: pembrolizumab + carboplatin/cisplatin + pemetrexed
Primary endpoint	Safety, tolerability	PFS	- PFS - OS
Status	Cohort A: FPI Q2 2023 Cohort B: FPI Q1 2024 - Recruitment completed Q4 2025	- FPI Q3 2024 - Recruitment completed Q4 2025	FPI Q4 2025
CT Identifier	NCT05789082	NCT06497556	NCT06793215

NSCLC: Non-small cell lung cancer

Giredestrant (SERD (3), RG6171, GDC-9545)

A selective estrogen receptor degrader or downregulator

Indication	Adjuvant ER-positive breast cancer (BC)	1L ER-positive metastatic breast cancer (mBC)	ET resistant ER+/HER2-negative breast cancer (BC)
Phase/study	Phase III lidERA Breast Cancer	Phase III persevERA Breast Cancer	Phase III pionERA
# of patients	N=4,100	N=978	N=1050
Design	ARM A: Giredestrant monotherapy ARM B: Tamoxifen or aromatase inhibitor	ARM A: Giredestrant + palbociclib ARM B: Letrozole + palbociclib	ARM A: Giredestrant + CDK4/6i ARM B: Fulvestrant + CDK4/6i
Primary endpoint	Invasive disease-free survival	Progression-free survival	Progression-free survival in ESR1m and ITT
Status	- FPI Q3 2021 - Recruitment completed Q3 2023 - Study met its primary endpoint at interim analysis Q4 2025 - Presented at SABCS 2025 - Filed in US Q1 2026 (FDA acceptance pending)	- FPI Q4 2020 - Recruitment completed Q1 2023 - Study did not meet primary endpoint Q1 2026	FPI Q4 2023
CT Identifier	NCT04961996	NCT04546009	NCT06065748

ER: Estrogen receptor; HER2: Human Epidermal growth factor Receptor; SABCS: San Antonio Breast Cancer Symposium

Giredestrant (SERD (3), RG6171, GDC-9545)

A selective estrogen receptor degrader or downregulator

Indication	post-CDK4/6 ER-positive/HER2-negative breast cancer (BC)	1L ER-positive/HER2-positive breast cancer (BC)	Grade 1 endometrial cancer
Phase/study	Phase III evERA	Phase III heredERA	Phase II endomERA
# of patients	N=373	N=812	N=45
Design	ARM A: giredestrant + everolimus ARM B: exemestane + everolimus	Induction Phesgo + taxane followed by maintenance with either: ARM A: Giredestrant + Phesgo ARM B: Phesgo	Giredestrant once a day (QD) on days 1 to 28 of each 28-day cycle for 6 cycles
Primary endpoint	Progression-free survival in ESR1m and ITT	Progression-free survival	Percentage of participants who have regression by 6 months
Status	- FPI Q3 2022 - Recruitment completed Q3 2024 - Study met its primary endpoint of PFS Q3 2025 - Presented at ESMO 2025 - Filed in US Q4 2025	- FPI Q2 2022 - Recruitment completed Q3 2025	- FPI Q2 2020 - Recruitment completed Q3 2024 - Presented at SGO 2026
CT Identifier	NCT05306340	NCT05296798	NCT05634499

ER: Estrogen receptor; ESMO: European Society for Medical Oncology; HER2: Human Epidermal growth factor Receptor; Phesgo: FDC of Perjeta and Herceptin for SC administration with Halozyme's rHuPH20/ Halozyme's human hyaluronidase; ITT: Intention to treat

NXT007 (FIXa x FX, RG6512)

Bispecific antibody which targets Factor IXa and Factor X

Indication	Hemophilia A		
Phase/study	Phase I/II (multiple-ascending dose)	Phase III (ZEBRHA 1)	Phase III (ZEBRHA 2)
# of patients	N=40	N=126	N=360
Design	Two loading doses of NXT007 SC (Q2W) followed by Q4W maintenance dosing	- H2H vs Factor VIII ARM A: Two loading doses, followed by Q4W maintenance of NXT007 ARM B: FVIII standard of care prophylaxis	- H2H vs Hemlibra ARM A: Two loading doses, followed by Q4W maintenance of NXT007 ARM B: Hemlibra prophylaxis
Primary endpoint	Safety	Annualized Bleed Rate (ABR) for Treated Bleeds Over the Main Study Treatment Period	Annualized Bleed Rate (ABR) for Treated Bleeds Over the Main Study Treatment Period
Status	- FPI Q4 2023 - Recruitment completed Q4 2024 - Data presented at ASH 2025	FPI expected Q2 2026	FPI expected Q2 2026
CT Identifier	NCT05987449	NCT07416526	NCT07416604

In collaboration with Chugai

ASH: American Society of Hematology; FIXa: Factor 9a; FX: Factor 10

Dirloctocogene Samoparvovec (SPK-8011QQ; RG6797)

Gene therapy for Hemophilia A

Indication	Moderately severe to severe Hemophilia A (>18yrs)
Phase/study	Phase IIb
# of patients	N=5
Design	Participants will receive an IV infusion of dirloctocogene samoparvovec on day 1
Primary endpoint	Safety
Status	FPI Q1 2026
CT Identifier	NCT07226206

Fenebrutinib (RG7845, GCD-0853)

Highly selective and reversible (noncovalent) bruton tyrosine kinase

Indication	Relapsing multiple sclerosis (RMS)		
Phase/study	Phase III FENhance 1	Phase III FENhance 2	Phase II (Biomarker study) FENopta
# of patients	N=746	N=751	N=109
Design	ARM A: Fenebrutinib twice daily oral ARM B: Teriflunomide once daily oral	ARM A: Fenebrutinib twice daily oral ARM B: Teriflunomide once daily oral	ARM A: Fenebrutinib ARM B: Placebo
Primary endpoint	Annualized relapse rate	Annualized relapse rate	Total number of new gadolinium-enhancing T1 lesions observed on MRI scans of the brain at 12 weeks
Status	- FPI Q1 2021 - Recruitment completed Q1 2024 - Primary endpoint met Q1 2026 - Presented at AAN 2026	- FPI Q1 2021 - Recruitment completed Q4 2023 - Primary endpoint met Q4 2025 - Presented at AAN 2026	- Data presented at EAN and ECTRIMS 2023 48-week OLE data presented at ECTRIMS 2024 96 week OLE data presented at CMSC 2025 and ECTRIMS 2025
CT Identifier	NCT04586010	NCT04586023	NCT05119569

AAN: American Academy of Neurology; CMSC: Consortium of Multiple Sclerosis Centers; EAN: European Academy of Neurology; ECTRIMS: European Committee for Treatment and Research in Multiple Sclerosis

Fenebrutinib (RG7845, GCD-0853)

Highly selective and reversible (noncovalent) bruton tyrosine kinase

Indication	Primary progressive multiple sclerosis (PPMS)
Phase/study	Phase III FENTrepid
# of patients	N=985
Design	▪ ARM A: Fenebrutinib twice daily oral ▪ ARM B: Ocrevus 2x300mg IV Q24W
Primary endpoint	▪ Time to onset of cCDP12
Status	▪ FPI Q4 2020 ▪ Recruitment completed Q2 2023 ▪ Primary endpoint met Q4 2025 ▪ Presented at ACTRIMS 2026
CT Identifier	NCT04544449

Prasinezumab (anti- α Synuclein; RG7935)

Anti-alpha-synuclein antibody under investigation for Parkinson's disease

Indication	Early-stage Parkinson's disease		
Phase/study	PASADENA Phase II	PADOVA Phase IIb	PARAISO Phase III
# of patients	316	586	N=900
Design	- PART 1: ARM A: Prasinezumab IV Q4W (low dose) ARM B: Prasinezumab IV Q4W (high dose) ARM C: Placebo Q4W - Part 2: COHORT A: Prasinezumab IV Q4W (low dose) vs Placebo Q4W COHORT B: Prasinezumab IV Q4W (high dose) vs Placebo Q4W - Part 3: All low dose and high dose participants to receive low dose prasinezumab IV Q4W for an additional 5 years	ARM A: Prasinezumab IV Q4W ARM B: Placebo Q4W - OLE: Participant to enter the OLE once double-blind tx period has been completed	ARM A: Prasinezumab IV Q4W ARM B: Placebo Q4W - OLE: Participant to enter the OLE once double-blind tx period has been completed
Primary endpoint	Change from baseline in movement disorder society-unified Parkinson's disease rating scale (MDS-UPDRS) total score (sum of Parts I, II, and III) at week 52	Time to Confirmed Motor Progression Event from BL to 28 days after last dose	Time to Confirmed Motor Progression Event (MDS-UPDRS Part III Score)
Status	- Recruitment completed Q4 2018 - Data presented at MDS & ADPD 2020-22 - OLE data presented at MDS 2023 and ADPD 2024	- Recruitment completed Q1 2023 - Primary endpoint missed, but numerical delay in motor progression and positive trends on multiple secondary and exploratory endpoints shown. - Data presented at ADPD 2025 and ADPD 2026	FPI Q4 2025
CT Identifier	NCT03100149	NCT04777331	NCT07174310

Tominersen (HTT ASO, RG6042)

Antisense oligonucleotide (ASO) targeting human HTT mRNA

Indication	Huntington's disease
Phase/study	Phase II GENERATION HD2
# of patients	N=300
Design	Patients aged 25 to 50 years with prodromal (very early subtle signs of HD) or early manifest HD ▪ ARM A: Tominersen 60mg Q16W via a lumbar puncture ▪ ARM B: Tominersen 100mg Q16W via a lumbar puncture ▪ ARM C: Placebo Q16W via a lumbar puncture
Primary endpoint	▪ Safety, biomarkers and efficacy
Status	▪ FPI Q1 2023 ▪ Recruitment completed Q4 2024
CT Identifier	NCT05686551

In collaboration with IONIS
 HD: Huntington's Disease; HTT: Huntingtin

Trontinemab (BS-anti-A β mAb, RG6102)

A novel Brainshuttle™ bispecific 2+1 monoclonal antibody targeting A β

Indication	Prodromal or mild to moderate Alzheimer's Disease	Early Alzheimer's Disease	
Phase/study	Phase I/II	Phase III TRONTIER 1	Phase III TRONTIER 2
# of patients	N=210	N=800	N=800
Design	PART 1 (dose escalation): Q4W trontinemab or placebo for 28w (5 dosing cohorts) PART 2 (expansion): Q4W trontinemab vs placebo for 28w (1.8mg/kg and 3.6mg/kg cohorts) PART 3 (PK/PD): Q4W trontinemab vs placebo (1.8mg/kg); Q12W trontinemab vs placebo (3.6mg/kg) PART 4 (open label extension): For all parts	Randomised 1:1 placebo-controlled study Induction: Q4W trontinemab (3.6 mg/kg) (7 doses) Maintenance: Q12W trontinemab (3.6 mg/kg) for an additional 12 months OLE: All switched to trontinemab from 18 months onward	Randomised 1:1 placebo-controlled study Induction: Q4W trontinemab (3.6 mg/kg) (7 doses) Maintenance: Q12W trontinemab (3.6 mg/kg) for an additional 12 months OLE: All switched to trontinemab from 18 months onward
Primary endpoint	- Part 1-4: Percentage of Participants with AEs - Part 3: Change From baseline in brain amyloid load (via PET)	Change in cognition and function based on the Clinical Dementia Rating – Sum of Boxes scale after 18 months of treatment	Change in cognition and function based on the Clinical Dementia Rating – Sum of Boxes scale after 18 months of treatment
Status	- FPI Q1 2021 - Data showing rapid, robust amyloid depletion presented at ADPD, CTAD 2024 and ADPD 2025	FPI Q3 2025	FPI Q3 2025
CT Identifier	NCT04639050	NCT07169578	NCT07170150

Sefaxersen (ASO factor B, RG6299)

Antisense oligonucleotide that targets factor B

Indication	IgA nephropathy (IgAN)
Phase/study	Phase III IMAGINATION
# of patients	N=428
Design	▪ ARM A: Sefaxersen SC at week 1, 3, 5 following Q4W dosing for 104 weeks ▪ ARM B: Placebo
Primary endpoint	▪ Change in UPCR at week 37 from baseline
Status	▪ FPI Q3 2023
CT Identifier	NCT05797610

In collaboration with IONIS

UPCR: Urine protein-to-creatinine ratio; ASO: Antisense oligonucleotide

Afimbikart (anti-TL1A, RG6631)

A monoclonal antibody targeting TL1A, blocking TH1 and TH17 pathways

Indication	Moderate to severe ulcerative colitis	Moderate to severe ulcerative colitis
Phase/study	Phase III AMETRINE-1	Phase III AMETRINE-2
# of patients	N=400	N=350
Design	ARM A: Afimbikart IV induction followed by afimbikart SC maintenance ARM B: Placebo IV followed by placebo SC maintenance	ARM A: Afimbikart IV induction ARM B: Placebo IV
Primary endpoint	Modified Mayo Score 0–2 (Stool Frequency Subscore = 0 or 1, Rectal Bleeding Subscore = 0, Endoscopic subscore = 0 or 1) at week 12 or week 52	Modified Mayo Score 0–2 (Stool Frequency Subscore = 0 or 1, Rectal Bleeding Subscore = 0, Endoscopic subscore = 0 or 1) at week 12
Status	- FPI Q3 2024 - Recruitment completed Q1 2026	FPI Q4 2024
CT Identifier	NCT06589986	NCT06588855

TL1A: Tumor necrosis factor-like cytokine 1A; TH: T helper cell

Afimkibart (anti-TL1A, RG6631)

A monoclonal antibody targeting TL1A, blocking TH1 and TH17 pathways

Indication	Moderate to severe Crohn's Disease	
Phase/study	Phase III SIBERITE-1	Phase III SIBERITE-2
# of patients	N=600	N=425
Design	- Treat-through design with no re-randomization after induction ARM A: Afimkibart IV induction followed by SC maintenance (high dose) ARM B: Afimkibart IV induction followed by SC maintenance (low dose) ARM C: Placebo IV followed by placebo SC maintenance	- Induction only ARM A: Afimkibart IV induction ARM B: Placebo IV
Primary endpoint	- Co-primary endpoints: - Clinical remission (CDAI <150) at w52 - Decrease in SES-CD from BL ≥50% at w52	- Co-primary endpoints: - Clinical remission (CDAI <150) at w12 - Decrease in SES-CD from BL ≥50% at w12
Status	FPI Q1 2025	FPI Q2 2025
CT Identifier	NCT06819878	NCT06819891

TL1A: Tumor necrosis factor-like cytokine 1A; CDAI: Crohn's Disease Activity Index; SES-CD: Simple Endoscopic Score for Crohn's Disease

Afimkibart (anti-TL1A, RG6631)

A monoclonal antibody targeting TL1A, blocking TH1 and TH17 pathways

Indication	Atopic dermatitis	Metabolic Dysfunction-associated Steatohepatitis (MASH)	Rheumatoid Arthritis (RA)
Phase/study	Phase II VELARITE	Phase Ib FIBROLITE	Phase IIb dRAvite
# of patients	N=160	N=50	N=160
Design	- ARM A: High dose afimkibart SC - ARM B: Med dose afimkibart SC - ARM C: Low dose afimkibart SC - ARM D: Placebo	Afimkibart IV at w0, w2, w6, w10 + afimkibart SC from w14-50	- ARM A: High dose afimkibart SC - ARM B: Low dose afimkibart SC - ARM C: Placebo
Primary endpoint	Percentage achieving EASI-75 Response ($\geq 75\%$ Improvement from baseline) at week 16	Percentage of participants with AEs from baseline to w52	Change from Baseline in Disease Activity Score-28 for Rheumatoid Arthritis with C-Reactive Protein (DAS28-CRP) at week 14
Status	- FPI Q2 2025 - Recruitment completed Q1 2026	- FPI Q2 2025 - Recruitment completed Q3 2025	FPI Q4 2025
CT Identifier	NCT06863961	NCT06903065	NCT07137598

TL1A: Tumor necrosis factor-like cytokine 1A; EASI-75: Eczema Area and Severity Index-75

Vamikibart (anti-IL-6, RG6179)

A monoclonal antibody that potently binds interleukin-6 (IL-6) cytokine

Indication	Uveitic macular edema (UME)	
Phase/study	Phase III MEERKAT	Phase III SANDCAT
# of patients	N=225	N=225
Design	ARM A: Vamikibart low-dose Q4W to week 12, followed by PRN ARM B: Vamikibart high-dose Q4W to week 12, followed by PRN ARM C: Sham control Q4W to week 12, followed by PRN	ARM A: Vamikibart low-dose Q4W to week 12, followed by PRN ARM B: Vamikibart high-dose Q4W to week 12, followed by PRN ARM C: Sham control Q4W to week 12, followed by PRN
Primary endpoint	Proportion of participants with ≥ 15 letter improvement from baseline in BCVA at week 16	Proportion of participants with ≥ 15 letter improvement from baseline in BCVA at week 16
Status	- FPI Q1 2023 - Recruitment completed Q2 2024 - Study met primary endpoint Q1 2025 - Data presented at AAO 2025	- FPI Q1 2023 - Recruitment completed Q4 2024 - Study did not meet primary endpoint Q3 2025, but nominally significant improvements seen across primary and key secondary endpoints - Data presented at AAO 2025
CT Identifier	NCT05642312	NCT05642325

AAO: American Academy of Ophthalmology; BCVA: Best corrected visual acuity; PRN: Pro re nata

Enicepatide (CT-388, RG6640)

Once-weekly subcutaneous injectable dual GLP-1/GIP receptor agonist

Indication	Overweight/obesity without type 2 diabetes	
Phase/study	Phase II	Phase III Enith 1
# of patients	N=450	N=2000
Design	enicepatide (low/med/high dose) vs. placebo	- Arm 1: Placebo - Arm 2: CT-388 dosing regimen 1 - Arm 3: CT-388 dosing regimen 2 - Arm 4: CT-388 dosing regimen 3
Primary endpoint	Efficacy: Percent change in body weight from baseline	Percent change in body weight from baseline at week 72
Status	- FPI Q3 2024 - Recruitment completed Q4 2024 - Study met primary endpoint January 2026	FPI Q1 2026
CT Identifier	NCT06525935	NCT07351045

GLP-1: Glucagon-like peptide-1; GIP: Glucose-dependent insulinotropic polypeptide

Enicepatide (CT-388, RG6640)

Once-weekly subcutaneous injectable dual GLP-1/GIP receptor agonist

Indication	Overweight/obesity with type 2 diabetes	
Phase/study	Phase II	Phase III Enith 2
# of patients	N=360	N=1600
Design	enicepatide (low/med/high dose) vs. placebo	- Arm 1: Placebo - Arm 2: Enicepatide dosing regimen 1 - Arm 3: Enicepatide dosing regimen 2 - Arm 4: Enicepatide dosing regimen 3
Primary endpoint	- Percent Change in Body Weight from baseline - Change in Glycated Hemoglobin (HbA1c) from baseline	Percent change in body weight from baseline at week 72
Status	- FPI Q4 2024 - Recruitment completed Q4 2025	FPI Q1 2026
CT Identifier	NCT06628362	NCT07351058

GLP-1: Glucagon-like peptide-1; GIP: Glucose-dependent insulinotropic polypeptide

CT-996 (GLP-1 RA, RG6652)

Once-daily oral small molecule GLP-1 receptor agonist

Indication	Overweight/obesity with or without type 2 diabetes	Obesity without type 2 diabetes	Glycaemic control trial with T2D participants
Phase/study	Phase I	Phase II	Phase II
# of patients	N=118	N=340	N=240
Design	Single ascending dose, multiple ascending dose, multiple part study, with low to high doses of CT-996 vs placebo up to 4 weeks	ARM 1: Placebo ARM 2-8: CT-996 with various uptitration schedules and step sizes towards 5 different maximum dosages	ARM 1: Placebo ARM 2: Commercially available incretin to be uptitrated in line with label ARM 3-9: CT-996 with various uptitration schedules and step sizes towards 5 different maximum dosages
Primary endpoint	Safety and tolerability	Percent change in body weight at week 30	Percent change in HbA1c at week 30
Status	- FPI Q2 2023 - Positive topline results at 4 weeks in people with obesity without type 2 diabetes July 2024, data presented at EASD 2024	- FPI Q3 2025 - Recruitment completed Q3 2025	- FPI Q3 2025 - Recruitment completed April 2026
CT Identifier	NCT05814107	NCT07081958	NCT07112872

EASD: European Association for the Study of Diabetes; GLP-1: Glucagon-like peptide-1; RA: Receptor agonist

Acmopatide (CT-868, RG6641)

Once-daily subcutaneous injectable dual GLP-1/GIP receptor agonist

Indication	Type 1 diabetes with BMI $\geq$ 27
Phase/study	Phase II
# of patients	N=96
Design	- ARM A: Acmopatide low dose - ARM B: Acmopatide medium dose - ARM C: Acmopatide high dose - ARM D: Placebo
Primary endpoint	Efficacy: Change in HbA1c from baseline
Status	- FPI Q4 2023 - Recruitment completed Q1 2025 - Study met primary endpoint Q1 2026
CT Identifier	NCT06062069

GLP-1: Glucagon-like peptide-1; GIP: Glucose-dependent insulinotropic polypeptide; RA: Receptor agonis; BMI: Body Mass index; HbA1c: Hemoglobin A1c

Emugrobart (RG6237, GYM 329)

Recycling and antigen-sweeping monoclonal anti-latent myostatin antibody

Indication	Obesity	
Phase/study	Phase Ib	Phase II GYMINDA
# of patients	N=30-36	N=234
Design	Cohort A (n=15-18): Single dose emugrobart 50mg SC Cohort B (n=15-18): Multiple dosing 180mg SC Q4W week + loading dose for first 3 doses	48w DB Tx period: Arm A: Tirzepatide + Placebo Arm B: Tirzepatide + GYM 329 (low SC Q4W) Arm C: Tirzepatide + GYM 329 (med SC Q4W) Arm D: Tirzepatide + GYM329 (high SC Q4W) - Extension: - Arm A-C switched to placebo - Arm D re-randomized to GYM329 Q4W or placebo
Primary endpoint	PK/PD, tolerability, safety	Percent (%) change in body weight from baseline at Week 48
Status	FPI Q2 2024	- FPI Q2 2025 - Recruitment completed Q3 2025
CT Identifier	NCT06965413	

emugrobart (GYM 329) in collaboration with Chugai

Pegozafermin (RG6881)

Fibroblast growth factor 21 (FGF21) analog

Indication	MASH F2-F3	MASH with compensated cirrhosis (F4)
Phase/study	Phase III ENLIGHTEN-Fibrosis	Phase III ENLIGHTEN-Cirrhosis
# of patients	N=1050	N=762
Design	- ARM A: Placebo - ARM B: Pegozafermin 30mg QW - ARM C: Pegozafermin 44mg Q2W	- ARM A: Placebo - ARM B: Pegozafermin 30mg QW
Primary endpoint	- Co-primary Endpoints: - Proportion of participants with one-point improvement in fibrosis with no worsening of MASH at 52 weeks - Proportion of participants with MASH resolution with no worsening of fibrosis at 52 weeks	Proportion of participants achieving fibrosis regression at 24 months
Status	FPI 2024	FPI 2024
CT Identifier	NCT06318169	NCT06419374

MASH: Metabolic Dysfunction-Associated Steatohepatitis

Roche Group development pipeline

Marketed products development programmes

Roche Pharma global development programmes

Roche Pharma research and early development (pRED)

Genentech research and early development (gRED)

pRED oncology development programs

Molecule	Indication	Phase	# of patients	Status	CT Identifier
Oncology					
mosperafenib (BRAFi (3), (RG6344))	solid tumors	I	292	FPI Q1 2022	ISRCTN 13713551
CEA212Pb PRIT (RG6375)¹	metastatic Colorectal Cancer (mCRC)	I	160	FPI expected Q2 2026	NCT07416552
PanRAS inhibitor (RG6505)	solid tumors	I	345	FPI Q2 2025	NCT06884618

pRED neurology development programs

Molecule	Indication	Phase	# of patients	Status	CT Identifier
Neurology					
Brainshuttle™-CD20 (RG6035)	multiple sclerosis	I	119	FPI Q3 2021	ISRCTN16295177 NCT05704361
nivegaceter (gamma-secretase modulator, RG6289)	Alzheimer's disease	II	255	FPI Q2 2024 Recruitment completed Q2 2025	NCT06402838
MAGL inhibitor (RG6182)	multiple sclerosis	I	Up to 36	FPI Q3 2023	
		II	360	FPI Q1 2026	NCT07282574
selnoflast* (NLRP3i, RG6418)	Parkinson's disease	Ib	57	FPI Q3 2022	NCT05924243
NME (RG6434)	neurodegenerative disorders	I		FPI Q4 2024	
HTT miRNA GT (RG6662)	Huntington's disease	I	part A: 8 part B:45	FPI Q2 2025	NCT06826612
HTT SNP ASO (RG6496) ¹	Huntington's disease	I	40	FPI Q4 2025	NCT07246941
APOE ASO (RG6627) ¹	Alzheimer's disease	I	38	FPI Q1 2026	NCT07234942

Partner: ¹IONIS, *molecule also in gRED development: Phase II in coronary artery disease

pRED immunology, ophthalmology and CVRM development programs

Molecule	Indication	Phase	# of patients	Status	CT Identifier
Immunology					
selnoflast* (NLRP3i, RG6418)	asthma	Ib	60	FPI Q1 2024	
CD19 x CD3 (RG6382)	SLE	I	70	FPI Q4 2023	NCT05835986
NME (RG6498)	IBD	I		FPI Q1 2026	
anti-TSLP:IL33 (RG6981)¹	COPD/Asthma	I	120	FPI Q1 2026	ISRCTN22936350
Ophthalmology					
VEGF x IL6 DutaFab (RG6209)	DME	I	176	FPI Q4 2022 LPI Q4 2025	NCT06847854 (PREGONDA)
NME (RG6327)	geographic atrophy	I		FPI Q3 2025	
CVRM					
NME (RG6622)	coronary artery disease	I		FPI Q1 2026	
Other					
zosurabalpin (Abx MCP, RG6006)	A. baumannii infections	I	204	FPI Q4 2020	NCT04605718

Partner: ¹Qyuns Therapeutics *molecule also in gRED development: Phase II in coronary artery disease
 Abx MCP: Antibiotic macrocyclic peptide

Roche Group development pipeline

Marketed products development programmes

Roche Pharma global development programmes

Roche Pharma research and early development (pRED)

Genentech research and early development (gRED)

gRED oncology development programs -1

Molecule	Indication	Phase	# of patients	Status	CT Identifier
Oncology					
cevostamab (anti-FcRH5 x CD3; RG6160)	R/R MM	I	355	FPI Q3 2017 LPI Q2 2023 Data presented at ASH 2020-2024	NCT03275103
	R/R MM	I	120	FPI Q2 2021	NCT04910568 (CAMMA1)
	BCMA-experienced R/R MM	I/II	140	FPI Q4 2022	NCT05535244
	R/R MM	Ib	~110	FPI Q3 2023 In combination with elranatamab	NCT05927571
	MM platform study	I/II	50	FPI Q4 2023 Multiple molecules and combinations	NCT05583617
autogene cevumeran (Individualized Neoantigen-Specific Therapy (iNeST); RG6180)¹	adjuvant PDAC	II	260	FPI Q4 2023	NCT05968326 (IMcode003)
CDK4/2i (RG6794, GDC-4198)	HR+ HER2- Breast cancer	Ib/II	~285	FPI Q4 2025	NCT07100106
CDK4i (RG6795, GDC-0587)	HR+ HER2- Breast cancer	I	120-140	FPI Q1 2026	NCT07214662
enzelkitug (anti-CCR8; RG6411)	solid tumors	I	163	FPI Q4 2022	NCT05581004

Partner: ¹ BioNTech; MM: Multiple myeloma

gRED oncology development programs -2

Molecule	Indication	Phase	# of patients	Status	CT Identifier
Oncology					
AR degrader (RG6537, GDC-2992)¹	mCRPC	I	~160	FPI Q2 2023	NCT05800665
NME (RG6468)	solid tumors	I	110	FPI Q4 2023	NCT06031441
KRAS G12D inhibitor (RG6620)	solid tumors with KRAS G12D mutations	I	410	FPI Q4 2024	NCT06619587
P-CD19 x CD20-ALLO1 CAR-T (RG6540)	NHL	I	120	FPI Q2 2024	NCT06014762
NME (RG6735)	heme tumors	I	6	FPI expected Q2 2026	

gRED neurology, immunology, ophthalmology development programs

Molecule	Indication	Phase	# of patients	Status	CT Identifier
Neurology					
P-CD19 x CD20-ALLO1 CAR-T (RG6540)	multiple Sclerosis	I	~60	FPI Q4 2025	NCT07008378
Immunology					
TMEM16A potentiator (RG6421, GDC-6988)	muco-obstructive respiratory disease	Ic	128	FPI Q4 2024	NCT06603246
p40 x TL1A bispecific (RG6730)¹	ulcerative colitis	IIb	224	FPI Q3 2025	NCT06979336 (SUNCREST)
P-CD19 x CD20-ALLO1 CAR-T (RG6540)	systemic lupus erythematosus	I	162	FPI expected Q2 2026	NCT06984341
Ophthalmology					
Anti-Tie2 Agonist (RG6351)	DME	II	~285	FPI Q1 2025	NCT06850922 (THMAES)
OpRegen (RG6501)²	geographic atrophy	IIa	60	FPI Q1 2023	NCT05626114
anti-LRP5 (RG6697)	DME	I	66	FPI expected Q2 2026	NCT07425522
CVRM					
selnoflast (NLRP3i)	coronary artery disease with high inflammatory risk	IIa	162	FPI expected Q2 2026	NCT07448038

Doing now what patients need next